

THE WELLINGTON HOSPITAL

HCA Healthcare UK • Centre for Orthopaedics & Sports Medicine

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INDEPENDENT MEDICAL SPECIALIST CONSULTATION REPORT

CONFIDENTIAL MEDICAL REPORT & PRE-AUTHORISATION TREATMENT PLAN

Subspecialty: Knee & Lower Limb Arthroplasty / Sports Arthroscopy

Patient Full Name: Mr Julian Marcus VANCE

Date of Birth: 22 March 1982 (Age: 44)

Hospital ID (MRN): WLL-910482-P

Residential Address: 14 Kensington Park Gardens, London, W11 3HD

Contact Phone: 07700 900812

Date of Consultation: 08 September 2026

Consultant: Mr Alexander Sterling, FRCS (Tr&Orth)

GMC Specialist Reg: 6140928

Billing Provider ID: SPEC-ORTHO-8821

Consultation Mode: Face-to-Face (Platinum Centre)

Private Medical Insurer (PMI): Bupa Global / UK

Pre-Authorisation Code: BUP-772914-AUTH

Policy / Membership No: BI-9844201-A

Scheme Group: Corporate Gold Comprehensive

Procedure Code (Proposed): OPCS-4: W8200 / W8500

Primary Diagnosis (ICD-10): M23.22

Dear Colleague / Medical Director (Bupa Medical Approvals),

Re: Initial Orthopaedic Assessment, Diagnostic 3.0T MRI Review, and Surgical Plan

I had the pleasure of reviewing Mr Julian Vance today in our outpatient consulting rooms at The Wellington Hospital. Mr Vance is a 44-year-old managing director and amateur competitive squash player who was referred under his private medical insurance scheme for an urgent subspecialty evaluation of sudden-onset, mechanically debilitating right knee dysfunction.

History of Presenting Complaint

- **Mechanism of Injury (28/08/2026):** Sustained a non-contact deceleration and pivoting injury to the right knee during a squash match. He felt an acute, audible "pop" localized to the medial joint line, followed by rapid onset of intra-articular swelling within 4 hours.
- **Mechanical Symptoms:** Complains of distinct mechanical locking in flexion between 20° and 30°, requiring manual manipulation to unlock, alongside frequent sensations of painful "catching" and subjective instability on descending stairs.
- **Impact on Function:** Unable to run, pivot, or bear weight comfortably without a single-point crutch. Current Visual Analogue Scale (VAS) pain score is 7/10 on weight-bearing and 2/10 at rest.
- **Prior Interventions:** Managed initially with RICE (Rest, Ice, Compression, Elevation) and oral naproxen 500 mg BD prescribed via his private concierge GP, with minimal symptomatic relief.

Past Medical, Surgical & Social Profile

- **Medical History:** Mild hypercholesterolaemia (diet-controlled); no history of inflammatory arthropathy, diabetes, or deep vein thrombosis (DVT).
- **Previous Surgery:** Elective right open inguinal hernia repair (mesh, 2018, uncomplicated).
- **Current Medications:** Naproxen 500 mg twice daily (oral, with meals), Omeprazole 20 mg once daily (gastro-protection), Paracetamol 1 g PRN.
- **Allergies / Intolerances:** Penicillin (develops urticarial rash; cephalosporins tolerated with caution).
- **Social History:** Non-smoker, consumes 6–8 units of alcohol weekly. Regular gym attendance and competitive racket sports.

Targeted Musculoskeletal Physical Examination

- **Inspection & Alignment:** Normal mechanical axis (neutral valgus/varus alignment). Moderate right knee joint effusion (Grade 2 patellar tap positive). No skin breaches or erythema.
- **Range of Motion (Active / Passive):** Right knee exhibits restricted active extension with a 10° springy block (flexion deformity secondary to mechanical entrapment). Flexion limited to 110° due to capsular distension and posteromedial pain (contralateral left knee: 0°–140°).
- **Palpation & Provocation Tests:**
 - **Medial Compartment:** Marked tenderness over the posterior horn of the medial meniscus. **McMurray's Test** positive with an audible and palpable painful clunk on external rotation in flexion.
 - **Lateral Compartment:** Completely non-tender over lateral joint line; McMurray's lateral test negative.
 - **Ligamentous Stability:** Anterior Drawer, Lachman's test (firm end-point, < 3 mm translation, Grade 0), and Pivot Shift tests are all negative. Medial and Lateral Collateral Ligaments intact without laxity in 0° and 30° flexion.
 - **Patellofemoral Joint:** Nil patellar apprehension, tracking is congruent, Clarke's test negative.
 - **Neurovascular Status:** Distal peripheral pulses (posterior tibial and dorsalis pedis) bounding and symmetrical. Sensation intact throughout the L2–S1 dermatomes. Calf compartments soft and non-tender.

Diagnostic Imaging Review (3.0T MRI Right Knee — 02/09/2026)

Anatomical Structure	Radiological Findings (The Wellington Diagnostics Centre)	Classification
Medial Meniscus	Complete, unstable complex "bucket-handle" tear of the posterior horn and body, with a 14 mm fragment displaced into the intercondylar notch.	Modified Outerbridge Grade III
Lateral Meniscus	Preserved triangular morphology; normal low signal intensity throughout.	Intact / Grade I
Cruciate Ligaments	ACL and PCL intact with homogeneous low signal and continuous fibres.	No ligamentous injury
Collateral Ligaments	MCL and LCL normal calibre; minimal periligamentous fluid.	Intact
Articular Cartilage	Preserved chondral thickness across femorotibial and patellofemoral joints.	Outerbridge Grade I
Osseous Structures	Focal bone marrow oedema over the medial tibial plateau (bone contusion).	Traumatic

Diagnostic Coding & Clinical Impression

- **Primary Diagnosis:** Traumatic Unstable Bucket-Handle Tear of the Medial Meniscus with Locked Right Knee.
- **Secondary Diagnosis:** Traumatic bone contusion, medial tibial plateau, right knee.
- **ICD-10 Code:** M23.22 (Derangement of meniscus due to past or current tear/injury, right knee).
- **Classification:** Displaced intercondylar notch meniscal entrapment necessitating timely operative liberation to prevent permanent meniscal deformation and chondral attrition.

Surgical Management Plan & PMI Pre-Authorisation Request

Given the fixed mechanical block to extension, persistent locking, and MRI confirmation of a displaced bucket-handle fragment, conservative physical therapy alone is contraindicated. Mr Vance is keen to proceed with operative repair to restore joint kinetics.

I have scheduled Mr Vance for emergency day-case arthroscopic surgery:

1. **Proposed Procedure:** Right Knee Arthroscopy, Reduction of Displaced Bucket-Handle Tear, and Meniscal Repair (All-Inside / Inside-Out Device Suturing) ± Partial Meniscectomy if non-viable.
2. **Clinical Coding for Billing:**
 - **OPCS-4 Code:** W8200 (Primary repair of meniscus of knee) or W8500 (Endoscopic partial excision of meniscus).
 - **Anaesthetic Code:** General anaesthesia with ultrasound-guided adductor canal peripheral nerve block.
3. **Admission Details:** Day-case admission at The Wellington Hospital South Building (Theatre Suite 4), planned for 12 September 2026. Anticipated length of stay: 1 day.
4. **Postoperative Rehabilitation:** Provision of a hinged knee range-of-motion brace (locked 0°–90°), 6 weeks of non-weight-bearing/touchdown crutch ambulation, and accelerated private sports rehabilitation physiotherapy (10 sessions, OPCS: A8440).

With kind regards,

Mr Alexander Sterling, BSc (Hons), MBBS, FRCS (Tr&Orth)

Consultant Orthopaedic & Trauma Surgeon

Clinical Lead for Sports Knee Reconstruction

GMC Registration: 6140928 | Bupa Consultant

Code: 03920194

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